

Q How is stillbirth dealt with?

Stillbirth occurs when a baby dies inside the uterus after 24 weeks of pregnancy. It is the term used when the baby would have been “viable,” or developed enough to have potentially survived (albeit with assistance) if he had been born living. Stillbirth may occur before labor, while the baby is in the uterus, or a baby may die during labor. In the US, about three babies per 1,000 births are stillborn.

Only around one stillbirth in 10 occurs as the result of a serious congenital abnormality, that is, a birth defect that has put the baby's life at risk. In most other cases where a cause can be found, stillbirth occurs as a result of a pregnancy complication rather than from the baby's genetic makeup.

They are often picked up at routine prenatal appointments, when your doctor can't detect a heartbeat. If you are in late pregnancy, you may have alerted her to the fact that you haven't felt your baby's usual pattern of movement. If your doctor is concerned, she'll immediately send you for an ultrasound. Two sonographers will be there to confirm your diagnosis. You'll be encouraged to have someone with you for support and to listen to what the doctor tells you, since it can be hard to take in at such a sad time.

If you have a known medical condition, such as preeclampsia, you'll be encouraged to have your labor induced right away. If you are in no immediate danger, you may be able to return home for a few days to give yourself a little time to come to terms with what's happened before going through labor. After the delivery, you will be given quiet time to hold your baby if you would like to. You will be given medicine to stop your milk from coming in. You'll be asked if you want for a postmortem to be done; this will be done only with your consent. Going to all prenatal appointments and reporting any changes in your baby's pattern or strength of movement reduces the risk of stillbirth.

POSSIBLE CAUSES OF STILLBIRTH

The chart below gives some of the possible causes of stillbirth, though in some cases a definite reason for the stillbirth is never established.

WHAT HAS GONE WRONG	WHAT DOES IT MEAN?
Preexisting conditions	Conditions such as kidney failure, high blood pressure, diabetes (prior to becoming pregnant), and other chronic illness can cause pregnancy complications, so you will be monitored closely during your pregnancy.
Maternal age	Mothers over 40 years old are more likely to have a stillbirth than women who are in their 20s. This is because becoming pregnant when you're older increases your likelihood of having other risk factors for stillbirth—including congenital or chromosomal abnormalities in the developing baby, gestational diabetes, and high blood pressure.
Lifestyle factors	Heavy drinking, smoking, and recreational drug use are all risk factors for stillbirth, because they restrict the baby's access to oxygen and nutrients. Obesity can also increase the risk.
Multiples births	Possible complications with multiples may result in stillbirths, but your babies will be monitored throughout your pregnancy to ensure their well-being.
Preeclampsia	This condition causes high blood pressure in the mother, which in turn reduces blood flow to the baby, starving him of oxygen.
Problems with the placenta (including abruption)	Your placenta is your baby's life-support system in the uterus. Your doctor will check the growth of your baby at your appointments. If your baby is smaller than expected it may be due to the poor function of the placenta. If it stops working for any reason, your baby is starved of the oxygen and essential nutrients he needs for life. Placental abruption (when the placenta comes away from the uterine lining) can be why the placenta might stop working. Over 50 percent of unexplained stillbirths are thought to be because of placental problems, but all the causes are not yet fully understood.
Hemorrhaging (including abruption)	Losing blood during pregnancy has the automatic consequence that your baby receives a restricted blood supply, increasing the risks of stillbirth.
Obstetric cholestasis	This is a rare liver condition that's been linked with stillbirth, although it's unclear why the two are related. Your doctor may suggest delivering your baby at 37 weeks if you suffer from obstetric cholestasis.
Maternal infection	Although the placenta and amniotic sac provide a barrier against many infections, in some cases bacteria and viruses can make their way into your baby's system. The most common culprits are infections that travel from the vagina into the uterus such as Group B streptococcus, E.coli, and enterococcus, but this rarely happens before your water breaks—contact your doctor at this point and he or she will monitor you. Many sexually transmitted infections, such as chlamydia and mycoplasma can also increase the risk of stillbirth.
Problems with the umbilical cord	If your baby becomes entangled in his umbilical cord during labor, or lies on it in such a way that the vein and arteries within it become compressed, he can restrict his oxygen supply. A prolapsed cord, which protrudes into the birth canal, is another possible cause of stillbirth.